Your Parent Needs Help—Now What?

A 30-Day Action Plan for Doctors, Documents, Home Safety, and Family Decisions

A note from the author

I wrote this book for the moment when you realize something has changed and nobody has told you what to do next. A few years ago, I lost my mother. Before that, I lived through the decisions my parents had to make about her care, and afterward I watched my dad slowly work out how to live alone. This is the book I wish our family had had.

Most advice for caregivers is either too general to act on or too long to read when you're exhausted. This book tries to be neither. Every chapter ends with something you can do today, and you don't have to read it in order. Start wherever your family is right now.

How this book works

This book takes you through the first 30 days of becoming your parent's care coordinator, then through the 60 days after that.

Every chapter has the same six parts, so you always know where you are:

1. **The frustration:** the moment you are probably in right now.
2. **What may be happening:** the pattern behind it.
3. **What to do now:** concrete steps, with day ranges.
4. **Exactly what to say:** scripts you can read aloud.
5. **A tool:** a checklist, worksheet, or printable you can use today.
6. **When professional help is appropriate:** who to call, and when.

Start at Chapter 1 and follow the days, or jump to the chapter that matches the problem in front of you. The days are a guide, not a deadline.

Each chapter also includes a short section called **One family's story**. The families you'll meet there (Dana and her father, Frank; Marcus and his mother, Ruth; and sisters Elena and Sofia Reyes and their mother, Carmen) are composites. Each is built from situations caregivers commonly describe. None is a real person, and any resemblance is coincidental. They're here so you can see the steps in this book play out in an ordinary, imperfect family.

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1. Why Is This Suddenly My Job?	1–3	My Role and Responsibilities Worksheet
2. Is This an Emergency or Just a Bad Week?	3–5	72-Hour Family Triage Checklist
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4. I Have a List—What Do I Actually Do With It?	11–14	Concern-to-Action Planner
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9. I Don't Have Time to Read a Textbook	30, then weekly	Weekly Care Dashboard and Emergency Handoff Sheet (printable PDF)
10. What Happens When This Gets Worse?	31–90 and beyond	Care Transition Roadmap and 90-Day Family Action Plan (printable PDF)

Figures and program rules in this book were checked on November 1, 2026, and each source and its date are given where the figure appears. A list of sources with as-of dates is at the back of the book. These figures change, so check the named source before relying on a number. This book is general information, not medical, legal, or financial advice.

Chapter 1: Why Is This Suddenly My Job?

Days 1–3 of your 30-day plan

1. The frustration

"I know something needs to change, but I don't know what I'm responsible for, or whether I'm overreacting."

You're sitting in your car in the work parking lot, rereading a text from your sister: "Have you seen Dad's mail? It's all over the kitchen table." You have. You saw it three weeks ago, along with the dent in his bumper, the expired milk in the fridge, and the story he told you twice in one phone call. You told yourself he was tired.

Now you're wondering whether this is a real problem or whether you're a worrier turning normal aging into a crisis. Underneath that is a harder question: if it is real, whose job is it? Nobody called a meeting or handed you a folder. You noticed, and noticing is how most people become caregivers.

You're in large company. In its 2025 report, AARP and the National Alliance for Caregiving estimated that 63 million Americans, nearly one in four adults, provide care for a family member or friend. About 59 million of them care for an adult, and the total is up about 20 million since 2015. Few of them applied for the role.

This chapter is the job description nobody gave you.

2. What may be happening

Aging rarely announces itself. It shows up as small things that each have a reasonable excuse, until the excuses stop adding up. One missed appointment is an incident. A missed appointment plus unopened mail plus a dented bumper is a pattern. Noticing a pattern isn't overreacting.

The numbers explain why waiting is risky. According to the CDC, one in four adults 65 and older reports falling each year, and falls are the leading cause of injury and of injury-related death in this age group. In a CDC survey of 2014 data, about 37% of those who fell reported an injury that needed medical treatment or limited their activity, and about half had not talked to a health care provider about the fall. Your parent may be falling more than you know.

One distinction matters right now: **speed**. Gradual change over months is a pattern to track. Sudden change is a medical question. Delirium is a sudden shift in attention, thinking, and alertness, and in older adults it can be the first sign of a medical problem. Infections, especially urinary tract infections and pneumonia, are a common cause, and so

are medications. The VA notes that delirium comes on quickly, over days, while dementia comes on slowly, over months to years. Confusion that appears suddenly isn't "just getting older." Call the doctor the same day.

Two traps catch almost every new caregiver:

Taking over. You see a problem and fix it, whether or not anyone asked. It feels efficient, but it erodes your parent's dignity and quietly makes you responsible for everything.

Staying out. "It's not my place" feels respectful, but it usually means waiting for a crisis, when options are fewer and costs are higher.

The way through is knowing who controls what:

- **Your parent** controls their own life, including where they live, whom they see, which treatment they accept, and how they spend their money. Adults are presumed capable of making these choices. Forgetfulness, or a decision you disagree with, doesn't change that.
- **Doctors** make medical judgments: diagnoses, treatment recommendations, and whether a person can understand and weigh a particular decision.
- **Professionals** (attorneys, financial advisors, care managers, home care agencies) control their own areas of expertise.
- **Family** controls what each person chooses to contribute. No sibling controls another.
- **You** control your own role: what you offer, what you decline, and how you organize the effort.

Notice what's missing from that list: deciding for your parent.

3. What to do now

Step 1: Name your role. Say it plainly: *I coordinate help. I don't run a life.* A coordinator gathers information, connects people, and follows up. Your parent decides.

Step 2: Write down what you've seen. Spend ten minutes listing specifics with dates. "Dad seems off" is hard for a doctor to use. "March 3: missed his cardiology appointment; March 10: three unpaid bills; March 14: told me the same story twice" gets attention.

Step 3: Sort the problem. For each issue, ask: *whose decision is this?* If the answer is "my parent's," your job is to inform and support, not to override.

Step 4: Set your limits before you're exhausted. Caregivers who skip this step tend to pay for it. In AARP and the National Alliance for Caregiving's 2025 report, nearly one in four caregivers provided 40 or more hours of care per week, about one in five reported

fair or poor health, and nearly half reported at least one major financial impact, such as taking on debt.

Step 5: Know what you're not going to get from Medicare. Many families assume it will pay for ongoing help. Medicare doesn't pay for long-term care, meaning help with bathing, dressing, and using the bathroom, whether at home, in assisted living, or in a nursing home. Short-term rehab after a hospital stay is a different category. For scale, CareScout's 2025 Cost of Care Survey (released March 2026) reported national medians of $35 an hour for an in-home non-medical caregiver, $74,400 a year for assisted living, and $114,975 a year for a semi-private nursing home room. Your area may differ. Knowing this now will change a lot of what you decide later.

Step 6: Tell your siblings. Send a short, factual update now, even if they're distant.

> **One family's story**
>
> Dana, 52, lives 20 minutes from her widowed father, Frank, 81. Her brother lives in Denver. Because she's closest, she becomes the default caregiver without anyone deciding it.
>
> When Dana finds a stack of overdue bills on Frank's kitchen table, she quietly starts paying them online. Frank discovers it and is furious that she went into his accounts. After a few days, she apologizes and tries again: "I'd like to help with the bills. Do you want me to, or would you rather we set up autopay together?" He chooses autopay. Dana realizes her job is to coordinate, not to take over.

4. Exactly what to say

To your parent, opening the conversation: "Dad, I'm not here to take anything over. I've noticed a few things, and I'd rather ask you than guess. Can I tell you what I've seen, and you tell me if I've got it wrong?"

When they say "I'm fine": "I believe you want to be fine, and I'm not saying you're not. I'd just feel better if we checked. Would you rather start with a doctor visit, or with me going through the mail with you?"

Offering a choice between two small steps keeps the decision in their hands.

To your siblings: "I want everyone in the loop. Here's what I've seen: [two or three dated specifics]. I'm willing to coordinate, but I can't do everything. Can we get on a call this week and divide this up?"

To anyone who assumes you'll handle it all: "I can take on [specific task]. I can't take on [specific task]. Let's figure out who can."

5. Tool: My Role and Responsibilities Worksheet

Takes about 20 minutes. Revisit monthly.

A. What I've noticed

Date	What I saw	Sudden or gradual?	Mention to doctor?

B. Who decides? Check one box per row.

Area	Parent	Doctor/professional	Family together	Me
Medical treatment				
Where they live				
Money and bills				
Daily routines				
My time and limits				

C. My role (circle one for each: *Do / Support / Delegate / Decline*)

- Doctor appointments: ____
- Bills and mail: ____
- Home safety: ____
- Daily check-ins: ____
- Updating family: ____

D. My limits

- Hours per week I can give: ____
- Money I can contribute: ____
- Tasks I won't take on: ____

E. The one person I'll update this week: _____

6. When professional help is appropriate

Call 911 for chest pain, trouble breathing, or signs of a stroke such as a drooping face, slurred speech, or sudden weakness.

Call the doctor the same day for sudden confusion, fever, a new fall, or medication mix-ups.

Call a geriatric care manager (usually a nurse or social worker) if the situation feels tangled and you need someone to assess needs and map options.

Call an elder law attorney if you're unsure about powers of attorney, health care directives, or protecting assets. Chapter 3 covers documents.

Call the Eldercare Locator at 1-800-677-1116 to find your parent's local Area Agency on Aging. It's a free public service of the Administration for Community Living.

Seek counseling or a caregiver support group if guilt, resentment, or exhaustion is building. Looking after yourself is part of the job.

You've named your role. Next comes your first hard judgment call: whether what you're seeing is an emergency or just a bad week.

Chapter 2: Is This an Emergency or Just a Bad Week?

Days 3–5 of your 30-day plan

1. The frustration

"Every problem feels urgent, and I can't tell what needs attention today versus next month."

It's Sunday, 8:40 p.m. Your mom's cough sounds worse than it did on Friday. Your dad's blood pressure refill is two days late. Your brother has texted, "Should we take her in?" A neighbor mentioned your dad seemed unsteady at the mailbox. All of it arrives at the same volume.

So you do one of two things. You rush to the ER over a cold, or you tell yourself it's probably nothing and go to bed, and then you lie awake. Both are what happens when you have worries but no system for sorting them.

The three-color system below is meant to fill that gap.

2. What may be happening

Part of why everything feels urgent is that you don't have a baseline yet. You don't know what "normal" looks like for your parent's walking, thinking, eating, and sleeping, so every change looks equally alarming.

The other part is that older adults don't follow the textbook, which means a bad week can be an emergency in disguise. Heart trouble in older adults is a good example. A 2022 American Heart Association scientific statement notes that heart attacks in people 75 and older are more likely to happen without chest pain, and may show up as shortness of breath, fainting, or sudden confusion. Infection works the same way. Older adults may not run a high fever with a serious infection. A urinary tract infection can cause sudden confusion without the usual urinary symptoms. That matters because infection can progress to sepsis, and in 2008 U.S. hospital data, adults 65 and older were 13 times more likely than younger adults to be hospitalized with it.

Medications are another hidden risk. A CDC-led study of 2007–2009 data from 58 hospitals, published in the New England Journal of Medicine in 2011, estimated nearly 100,000 emergency hospitalizations a year for adverse drug events in adults 65 and older. Four kinds of medicine (warfarin, insulin, antiplatelet drugs, and diabetes pills) accounted for 67% of them, and about 66% involved unintentional overdoses.

The reverse is also true. Most bad weeks are just bad weeks. Your job is to sort quickly and consistently, so you act fast on the real emergencies without letting every worry run your life. You don't need to be a nurse to do that.

A note: this chapter is a guide, not a diagnosis. When you're unsure, call 911. The dispatcher will help you decide.

3. What to do now

Use three colors. For anything that worries you, ask: *Is this sudden? Could it cause serious harm in the next few hours? Is it different from my parent's normal?*

🔴 RED: Call 911 now

- **Stroke signs (BE FAST):** sudden trouble with balance or vision, facial drooping, arm weakness, or slurred speech. Call 911 even if the symptoms go away. Note the time they started. Don't drive your parent yourself, because paramedics can alert the hospital so the stroke team is ready.
- **Chest pressure, unexplained trouble breathing, or sudden confusion with dizziness**

- **Suspected sepsis:** an infection plus confusion, fast breathing, extreme fatigue, clammy skin, or severe pain.
- **A fall with a head strike while on blood thinners.** Seek emergency care even if symptoms seem mild, and call 911 for any weakness, numbness, slurred speech, or new confusion.
- **A fall where they can't get up,** or have severe pain or an obvious deformity
- **Talk of suicide with a plan or means.** Call 911 if someone may be in imminent danger.

🟡 YELLOW: Call a clinician today

- **A sudden change in thinking or alertness** without red-flag signs. This is the delirium pattern from Chapter 1.
- **Any fall,** even without injury. A head bump without blood thinners still deserves a same-day call, and the doctor may want to see your parent in person.
- **A medication mix-up:** double doses, skipped insulin or blood thinners, wrong pills. Call the pharmacist or Poison Help. The free, confidential line, 1-800-222-1222, is open around the clock and staffed by nurses, pharmacists, and doctors. HRSA says people usually get the help they need over the phone.
- **Not eating or drinking for a day or more,** new urinary symptoms, or a wound that's red, warm, or worsening
- **Possible exploitation or mistreatment.** You don't need proof to report to Adult Protective Services. For a suspected scam, the National Elder Fraud Hotline is 1-833-372-8311.
- **Hopelessness, giving things away, or saying goodbye.** Dialing or texting 988 connects family and caregivers, not just the person in crisis, to counselors around the clock. In 2024, men 75 and older had the highest suicide rate of any age group, 40.1 deaths per 100,000, according to the National Center for Health Statistics.

🟢 GREEN: Goes into the 30-day plan

Piled-up mail, forgetfulness that has crept in over months, a cluttered house, hearing or vision decline, a single missed appointment, loneliness, sibling friction, no legal documents. Green doesn't mean ignore. It means *log it and schedule it.*

The tiebreaker: when in doubt, go one color up. Being wrong in that direction costs you an embarrassing phone call. Being wrong the other way can cost far more.

> **One family's story**
>
> Marcus, 46, lives 700 miles from his mother, Ruth, 78, who lives alone. He sees her twice a year and relies on phone calls and a helpful neighbor.
>
> On a Sunday, the neighbor calls: Ruth seems confused and keeps asking what day it is. Marcus assumes her memory is finally slipping and plans to raise it at her next checkup. Then he remembers that this came on suddenly, which makes it yellow, not green. He calls her doctor's after-hours line. It's a urinary tract infection, and within a week of antibiotics Ruth is herself again.

4. Exactly what to say

To a 911 dispatcher: "My father is 82. Since about 2:15 he's had slurred speech and his left arm is weak. He takes a blood thinner. We're at [address]."

To the ER triage nurse: "Mom was walking and talking normally yesterday. Today she's confused and sleepy. She had a urinary infection in the spring. I'm concerned about sepsis." Naming sepsis out loud may prompt staff to check for it.

To the doctor's office (yellow): "I'm calling about my mother, [name, birthdate]. Since yesterday she's been more confused than usual, which is a sudden change for her. She takes [medications]. I'd like her seen today. If that's not possible, what should we do?"

To a parent who says "Don't make a fuss": "I'm not overruling you. I'm calling because this is the kind of thing doctors want to hear about right away. If I'm wrong, we've lost ten minutes."

To a parent refusing 911 during a red-flag moment: "I know you don't want to go. I'm having the paramedics check you out. You can decide what happens after they're here."

To your siblings, afterward: "Update: [what happened, when]. [Who we called, what they said]. Next step is [X]. Logged in the shared file."

5. Tool: The 72-Hour Family Triage Checklist

Hour 0–1: Scan.

- [ ] Check every red flag above. Any yes means call 911.
- [ ] Write down the exact time symptoms started.

Hours 1–24: Sort.

- [ ] List every worry and mark it 🔴 🟡 🟢.

- [ ] Make every yellow call today.
- [ ] Tell one sibling what you decided.

Hours 24–72: Build your safety net.

- [ ] Save these in your phone and your parent's: 911, doctor, pharmacy, Poison Help (1-800-222-1222), 988, Eldercare Locator (1-800-677-1116).
- [ ] Make a one-page **Grab Sheet:** full name, birthdate, address, allergies, diagnoses, every medication and dose, doctors' numbers, insurance, emergency contact, and a line on their baseline (how they normally walk, talk, and think). Keep copies in your phone and on the fridge.
- [ ] Ask the pharmacy for a printed medication list.
- [ ] Identify who lives closest and who has a key.
- [ ] Start the log below.

Incident and Symptom Log

Date/time	What happened (facts only)	Change from normal?	Who I contacted	What they said	Follow-up due

Write what you saw, not what you fear. "Dropped her fork twice at lunch, slurred 'Thursday'" is useful. "Seemed off" isn't.

6. When professional help is appropriate

- **Any red flag:** 911. Don't wait to see if it passes.
- **Yellow:** the doctor's office the same day. If there's no appointment, ask for the nurse line or an urgent care referral.
- **Medication confusion:** a pharmacist can review every prescription and supplement, and Poison Help answers urgent questions.
- **Several yellows in a month:** that's a pattern. Ask the doctor about a geriatric assessment, or consider a geriatric care manager.
- **Suspected abuse, neglect, or severe self-neglect:** contact Adult Protective Services in the state where your parent lives. The Eldercare Locator can help you find the right office.
- **You:** if you're not sleeping, a counselor or caregiver support group is part of the plan.

If you only do one thing this week, save the phone numbers and write the Grab Sheet.

Chapter 3: Everyone Says "Get Organized," But Organize What?

Days 6–10 of your 30-day plan

1. The frustration

"The advice sounds like common sense, but nobody tells me exactly what information I need."

A friend says, "You just need to get organized." A website says, "Gather important documents." So you buy a three-ring binder, write MOM on the spine, and sit down with it. Then you stall. Which documents? From where? Does "important" include the electric bill? The binder stays empty, and now you feel behind on something you can't even define.

The advice is fine as far as it goes. It just stops too soon. What follows covers seven specific areas, what to find in each, and how to find it without turning every visit into a deposition.

2. What may be happening

Generic advice fails in three ways. It doesn't say what to collect, it treats every gap as equally urgent, and it assumes you can simply ask. Your parent may not know where things are, may be embarrassed that they don't, or may hear "Where's your will?" as "I'm planning your funeral."

You're building two things:

A picture. That means facts (names, numbers, documents) plus observations of how your parent actually functions. Professionals need both. "Dad's medications" is vague. "Dad takes eleven pills a day from three pharmacies" is something a doctor can act on.

A ranking. You won't fix seven areas in a month, so you need to know which three gaps could do the most harm soonest.

One area runs on a clock the others don't. Legal documents must be signed while your parent can still understand them. A dementia diagnosis doesn't automatically bar someone from signing, because what matters is capacity at the time, but if someone becomes incapacitated without a power of attorney, a court may have to appoint a guardian. That's why legal paperwork gets early attention even when everything else looks calm.

3. What to do now

The seven areas

1. Health. Collect diagnoses, doctors' names and numbers, and insurance cards (Medicare, any supplement or Advantage plan, drug plan). Get written permission to talk to the doctors. Under the HIPAA privacy rule, providers aren't required to share information with family, though they may if you're involved in care and your parent doesn't object (HHS). Even if a provider can't tell you anything, nothing stops you from telling them what you've seen.

2. Medications. List every prescription, over-the-counter product, and supplement. In a 2020 University of Michigan National Poll on Healthy Aging of adults 50 to 80, about two in ten took five or more prescription medicines, and only 29% of those people had ever had a comprehensive medication review.

3. Daily functioning. Track two lists. The basic activities are feeding, continence, transferring, toileting, dressing, and bathing. The second list covers meals, shopping, medications, money, phone use, and transportation. Pay attention to the second list, because these more complex tasks typically decline before basic self-care does. Missed bills and pills often show up before bathing trouble.

4. Home safety. Walk through as a guest. CDC says falls are often caused by hazards that are easy to overlook but easy to fix. Look for loose rugs, clutter, stairs without handrails on both sides, a dark path from bed to bathroom, and no phone within reach at floor level.

5. Transportation. Is your parent still driving? Look for new dents, tickets, or getting lost. The CDC reports that about 9,100 older adults were killed in crashes in 2022, roughly 25 a day. The Insurance Institute for Highway Safety says the higher death rates for drivers 70 and older are mainly due to greater vulnerability to injury, not to being in more crashes. So this is about protecting your parent, not blaming them. Also ask who gets them to appointments if they stop driving.

6. Finances. Find income sources, regular bills, accounts, and where statements are kept. Watch for unopened mail or unusual withdrawals. In 2025, people 60 and older reported about $7.7 billion in fraud losses to the FBI, roughly 60% more than in 2024 (FBI figures as reported by AARP in April 2026). A "trusted contact" at the bank is one safeguard. A trusted contact generally can't make trades or decisions on the account. They're the person the institution can call if it sees signs of exploitation, and details vary by institution.

7. Legal documents. You're looking for a health care power of attorney, a financial power of attorney, a living will, a will, and the location of the originals. These are separate documents, because a financial power of attorney differs from a health care power of attorney, which covers only medical decisions. Gaps are common. A 2017 Health Affairs review of 150 studies conducted from 2011 to 2016 found that 36.7% of U.S. adults had completed an advance directive, and even among those 65 and older, the figure was 45.6%.

Finding missing information without interrogating

- **Ask "where," not "what."** "Where do you keep your insurance cards?" is easier than "What insurance do you have?"
- **Borrow a reason.** "I'm updating my own paperwork and realized I don't have yours."
- **Look, don't inspect.** Notice the mail pile, the pill bottles, the fridge, and the car, then write it down in the car afterward.
- **Go along.** Ask to join one appointment, and ask the pharmacy for a printed medication list.
- **One question per visit.** Small asks feel like help. A list of twenty feels like an audit.
- **Use other witnesses.** Siblings, neighbors, and longtime friends often know things your parent hasn't told you.

Naming your three biggest risks

Rate each area 🟢 fine for now, 🟡 needs a plan this month, or 🔴 likely harm within weeks. Then pick no more than three, using these questions. Could this cause serious harm soon? Is it hard to undo? Is there a deadline, like the capacity window? Those three drive your 30-day plan. Everything else waits.

Turning observations into notes for professionals

Write what happened, when, and how often, without a diagnosis. Compare "Mom is forgetful" with "Since March, Mom has missed three appointments, paid the water bill twice, and called twice in one evening to ask the same question." The second version gives a doctor something to work with. Keep it to one page and attach the medication list.

> **One family's story**
>
> Carmen Reyes, 84, lives near her daughter Elena, who does most of the hands-on help. Her other daughter, Sofia, lives across the country.
>
> Helping Carmen look for her insurance card, Elena comes across her mother's power of attorney. It was signed in 1998 and names Elena's father, who died six years ago. Nobody else is named as a backup. The sisters book an elder law attorney that month, while Carmen can still understand and sign new documents.

4. Exactly what to say

About papers: "Mom, I'm getting my own paperwork in order, and it made me realize I don't know where yours is. You don't have to show me anything. Could you just tell me where you keep it, so I'd know where to look if something happened?"

About the bank: "I'm not asking about your money. There's an option called a trusted contact. I'd have no access to your accounts. The bank would only call me if something looked like a scam. Would you be okay with that?"

About medications: "Would you do something with me? We put every bottle and supplement in a bag and let the pharmacist look it over. Your plan may even cover it."

About doctors: "Would you sign a form so Dr. Lee can talk to me and I can see your portal? You'd still make every decision. I'd just stop having to guess."

When they say "That's private": "That's fair, and I won't push. Can we agree on one person who knows where everything is if you're ever in the hospital? It can be me, your sister, or your attorney."

To a professional: "I've put together a one-page summary of what I've noticed. I'm not asking you to diagnose anything. I'd like to know what you think we should check first."

5. Tool: Seven-Area Aging Parent Assessment

Complete over several visits, not in one sitting. Rate each area 🟢 🟡 🔴.

1. Health

- [ ] Diagnoses list
- [ ] Doctors and phone numbers
- [ ] Insurance cards copied
- [ ] Last wellness visit date

- [ ] Permission/portal access

Rating: ___ Notes: ___

2. Medications

- [ ] Every prescription, over-the-counter product, and supplement listed
- [ ] Number of pharmacies
- [ ] Missed or doubled doses
- [ ] Pill organizer in use
- [ ] Pharmacist review done

Rating: ___ Notes: ___

3. Daily functioning (Independent / Needs help / Can't)

Eating ___ Bathing ___ Dressing ___ Toileting ___ Transferring ___ Continence ___

Meals ___ Shopping ___ Medications ___ Money ___ Phone ___ Transportation ___

Rating: ___ Notes: ___

4. Home safety

- [ ] Rugs and clutter
- [ ] Stair lighting and rails
- [ ] Lit path to bathroom
- [ ] Grab bars/non-slip surfaces
- [ ] Phone reachable from floor
- [ ] Working smoke alarms
- [ ] Fresh food in fridge

Rating: ___ Notes: ___

5. Transportation

- [ ] Still driving?
- [ ] New dents, tickets, or getting lost
- [ ] Vision/hearing checked
- [ ] Backup rides identified
- [ ] Appointment transport

Rating: ___ Notes: ___

6. Finances

- [ ] Income sources
- [ ] Regular bills and autopay
- [ ] Unopened or past-due mail
- [ ] Unusual withdrawals, gifts, or new "friends"
- [ ] Trusted contact on accounts
- [ ] Who knows where statements are

Rating: ___ Notes: ___

7. Legal documents

- [ ] Health care power of attorney
- [ ] Financial power of attorney
- [ ] Living will
- [ ] Will
- [ ] Originals located
- [ ] Doctor-sharing authorization

Rating: ___ Notes: ___

My Top Three Risks

Risk	Why it matters	First action	By when
1.			
2.			
3.			

One-Page Summary for Professionals

Name and birthdate: ___ | Top three concerns (dated, specific): ___ | What changed, and when: ___ | Medication list attached [ ] | My questions: ___

6. When professional help is appropriate

Laws vary by state, and this is general information, not legal advice.

- **Primary care doctor:** bring your one-page summary. Ask for the Medicare annual wellness visit. It costs nothing once every 12 months if the provider accepts Medicare's approved amount, and it includes a cognitive assessment. If impairment is suspected, Medicare covers a separate, more thorough cognitive visit.
- **Pharmacist:** ask about a comprehensive medication review, or do the "brown bag" version with every bottle.
- **Elder law attorney:** call soon if documents are missing, especially if memory is a concern. Capacity is time-sensitive.
- **Occupational therapist or geriatric care manager:** for a professional home-safety assessment or when the picture feels too tangled to sort alone.
- **Bank or financial advisor:** to add a trusted contact. For a suspected scam, the National Elder Fraud Hotline is 1-833-372-8311.
- **Adult Protective Services:** if you suspect exploitation or neglect. As in Chapter 2, you don't need proof.

Take the whole week for this. Three risks and a one-page summary are enough to move on.

Chapter 4: I Have a List—What Do I Actually Do With It?

Days 11–14 of your 30-day plan

1. The frustration

"I understand the problems. I still don't know what action to take Monday morning."

It's Monday, 7:15 a.m. Your coffee is going cold next to the assessment from Chapter 3. You've rated seven areas. Your three biggest risks are circled. You have twenty minutes before your first meeting, and you stare at a list that says things like *fix medications, make the house safer, get legal documents.*

Each item is a project, not a task. So you do what most people do, which is nothing, or something random like a Saturday spent rearranging your mother's pantry while the unpaid bills sit in the drawer.

What you need is a list of actions small enough to start today.

2. What may be happening

Lists freeze people when the items are the wrong size. "Fix Dad's medications" has no first step. "Photograph every bottle label on Sunday" does. A concern is a worry. An action is something you could do with your hands or your phone in under thirty minutes.

Specific plans work better than good intentions. A 2006 analysis by Gollwitzer and Sheeran, covering 94 independent tests, found that plans spelling out the when, where, and how of acting in advance had a medium-to-large effect on goal attainment. That's why every action in this chapter gets an owner, a date, and a trigger.

Two other forces work against you. The first is that medical appointments, where many of your actions run, leak information. A 2003 review in the Journal of the Royal Society of Medicine found that patients forget 40 to 80% of the medical information they are given almost immediately, and that almost half of what they do remember is incorrect. The second is that referrals leak too. In a study of one large health system covering July 2015 to June 2016, only about 35% of referral scheduling attempts ended in a documented completed appointment. That was a single system, but the point stands: if nobody tracks the referral, it can quietly disappear. Often that somebody is you.

Finally, don't try to repair everything at once. Your parent can absorb only so much change, and you can only do so much before you burn out. Doing things in order is how they actually get finished.

3. What to do now

Step 1: Convert each concern into a next action. A good next action is physical, visible, and under thirty minutes.

Concern	Next action
"Dad's medications are a mess."	Photograph every bottle label Sunday. Ask the pharmacy for a printed list.
"Mom may not be safe in the shower."	Ask her doctor's office for an occupational therapy home-safety referral.
"Bills are piling up."	Sit with Mom for 30 minutes and open one week's mail together.
"There are no legal documents."	Call two elder law offices and ask about fees and the first opening.

Concern	Next action
"Dad's driving worries me."	Ride along Thursday and write down three specific observations.

Step 2: Give every action an owner, a date, and a trigger. The owner is one name, not "we." Add a trigger: *"When I park at Mom's on Sunday, I'll photograph the bottles."* Remember Chapter 1, too. If the action involves your parent's life, your parent's role is to agree or help, not just to be informed.

Step 3: Choose the first three. Start from your top three risks, then filter. Can it be done in two weeks? Will your parent agree or at least not object? Can it be undone? Pick one action that's easy, because an early win builds your parent's trust. Everything else goes on a **parking lot** list with a review date. Keep no more than three actions active at once.

Step 4: Prepare for appointments. National Institute on Aging guidance on talking with your doctor is consistent. Make a prioritized list and bring up the biggest concerns first. Don't save what's on your mind for the end of the visit. Bring a list of every medication and supplement, or put the bottles in a bag. Bring a family member or friend who can take notes, and tell them in advance what you want from the visit. Also bring eyeglasses and hearing aids. Send your top three concerns through the patient portal beforehand if you can, and ask whether you should book a longer visit.

In the room, use teach-back. Repeat back what you think you heard. "Do you understand?" doesn't count, because real teach-back asks people to explain in their own words, not answer yes or no. If time runs out, ask to follow up by phone or email, book another visit, or speak with a nurse.

Step 5: Track every referral until the loop closes. A referral isn't done when it's ordered. It's done when the specialist has seen your parent and the report has reached the primary doctor. First, find out whether the plan requires one at all. Original Medicare doesn't require a referral to see a specialist, but Medicare Advantage plans may require a referral and prior authorization. Rules shift. For example, UnitedHealthcare announced in September 2025 that, starting January 1, 2026, most of its Medicare Advantage HMO and HMO-POS members would need a primary care referral for certain specialist services. Then, within three business days, call the specialist to confirm receipt, book the date, and log it.

Step 6: Hold a 20-minute weekly review. Every Sunday, check what's done, what's stuck, and whether anything on the parking lot can move up. One action finished means one parking-lot item can enter. If something becomes yellow or red, apply Chapter 2.

> **One family's story**
>
> Dana's list says "fix Dad's medications," and for three weeks nothing happens. When she shrinks it to "photograph every bottle on Sunday," she gets it done in fifteen minutes and finds that Frank uses three pharmacies and has two prescriptions for nearly the same blood pressure drug. After one pharmacist review, he's down to a single pharmacy and four fewer pills a day.

4. Exactly what to say

To your parent, proposing a start: "I don't want to change everything. There are three things on my list: the medications, the bathroom, and the bills. Which one would feel least intrusive to start with?"

To a sibling, assigning an owner: "Can you own the insurance piece? Call Mom's plan and find out whether she needs referrals or prior authorization. Can you finish by Friday and text me what you learn?"

To the doctor's office, before the visit: "I'm bringing my father Thursday. We have three concerns: [one, two, three]. Could you let the doctor know ahead of time, and tell me if we should book a longer visit?"

In the exam room, before leaving: "Let me make sure I have this right: [repeat the plan]. What should we watch for, and when should we call?"

To a referral desk: "I'm calling to confirm you received the referral for [name, birthdate] from Dr. [X] on [date]. What's the first available appointment, and will you send your notes back to Dr. [X]?"

To anyone pressing you to do more: "That's on the parking lot until [date]. We're finishing three things first."

5. Tool: The Concern-to-Action Planner

Part A: Action Plan

#	Concern (from Ch. 3)	Color 🟢🟡🔴	Next action (under 30 min)	Owner	Parent's role	Due	"Done" means	Active or Parking Lot
Example	*Medication confusion*	🟡	*Photograph bottles; request pharmacy list*	*Me*	*Agrees to show bottles*	*Sun.*	*List in phone*	*Active*

#	Concern (from Ch. 3)	Color 🟢🟡🔴	Next action (under 30 min)	Owner	Parent's role	Due	"Done" means	Active or Parking Lot
1								
2								
3								

Part B: Appointment Prep Sheet

Date/doctor: ___ | Who's attending / who speaks first: ___

Top 3 concerns: 1. ___ 2. ___ 3. ___

What changed and when: ___

- ☐ Medication list attached
- ☐ Glasses/hearing aids

After the visit: Decisions made: ___ | Tests or referrals ordered (who sends?): ___ | Medication changes: ___ | Call us if: ___ | Next visit: ___

- ☐ Teach-back done

Part C: Referral Tracker

Referral	Ordered (date, by whom)	Plan requires authorization?	Specialist and phone	Receipt confirmed	Appointment date	Report back to primary doctor?	Follow-up due

Part D: Sunday 20-Minute Review

- ☐ What got done?
- ☐ What's stuck, and why?
- ☐ Any new yellow or red?
- ☐ What moves off the parking lot?
- ☐ Who needs an update?

6. When professional help is appropriate

- **Insurance and referral questions:** call your State Health Insurance Assistance Program. It provides free, objective counseling to Medicare-eligible people, their families, and caregivers, at 1-877-839-2675.
- **The doctor's office:** ask whether the practice has a care coordinator or nurse navigator who can track referrals.
- **A geriatric care manager:** consider one when you have more moving parts than you can track, such as several specialists, a recent hospitalization, or family conflict.
- **A hospital discharge planner or social worker:** if your parent has just been discharged, use this person before you go home.
- **A caregiver support group or counselor:** if you're missing your own appointments or sleep, tell someone. Your capacity is part of the plan.

Get three actions moving by Day 14. Everything else can wait on the parking lot.

Chapter 5: How Do I Bring This Up Without Starting a Fight?

Days 15–21 of your 30-day plan

1. The frustration

"Every conversation ends with 'I'm fine,' 'It's none of your business,' or both."

You rehearsed in the car. "Mom, I've been thinking..." Before you finish, she says "I'm fine," and the kitchen gets colder. You push. She says, "It's none of your business." Then you hear yourself say, "Well, somebody has to." Ten minutes later you're both angry, nothing has changed, and you've become the villain in her version of the story.

Most of the time, the trouble is the timing, the order you say things in, and a few phrases that land as attacks. Those are fixable.

2. What may be happening

What you say is not what they hear. You say "I'm worried." They hear "You're losing control." In AARP's 2024 Home and Community Preferences Survey, 75% of adults 50 and older said they wish to remain in their current homes, so every offer of help can sound like the first step toward leaving. Driving carries the same weight.

Pushing backfires. Motivational interviewing calls the urge to fix someone the "righting reflex." When you argue for change, an ambivalent person tends to take up and defend the opposite side. The alternative is to ask permission before giving advice.

Sometimes it isn't stubbornness. In dementia, people may lack awareness of their illness. When short-term memory fails, a person may not remember that they can't remember. The refusal is a symptom, not intentional denial. Facts won't win that argument. A method called LEAP (Listen, Empathize, Agree, Partner) is a non-confrontational approach designed to build trust.

Tone matters more than you'd think. Baby-talk-style "elderspeak" can backfire. In a small 2021 hospital study published in Innovation in Aging, a 15-percentage-point drop in staff elderspeak was linked to 62% lower odds that a person with dementia would reject care. The study involved just 16 patients and 53 staff members at one hospital, so treat it as a signal, not proof.

You may not be the right messenger. Sometimes the best move is to hand the message to someone your parent trusts, such as their doctor.

3. What to do now

1. Start early, small, and private. Pick a calm time, not a holiday dinner and not when either of you is tired. Take one topic. The Alzheimer's Association advises having the first conversation as soon as possible, and notes that the first attempt may not succeed, so write down what happened and what to change. Waiting is common. In a 2021 survey for the Alzheimer's Association and the Ad Council, only 44% of Americans said they would talk to a loved one right away about seeing a doctor after noticing cognitive decline.

2. Use a four-part formula: *your intention, one specific observation, a permission question, a small choice.* ("I want you to stay in this house. I noticed the stairs light is out. Can I fix that this week, or would you rather call the electrician?")

3. Use the LEAP loop. Listen without interrupting. Empathize ("That would frustrate me too"). Agree on a goal ("We both want you home"). Then partner on a step.

4. Borrow authority. Ask the doctor to raise the issue, or suggest an assessment instead of a verdict.

5. When they refuse, don't escalate. Say you'll revisit it, set a date, and log what happened. Refusal on a green topic can wait. A red flag from Chapter 2 can't. If safety is at stake, you may need to act through a doctor, Adult Protective Services, or the DMV even without agreement.

6. Topic by topic:

- **Driving.** Start before it's a problem, and don't open with scary risk statistics. Try a starter like "Driving isn't what it used to be." Line up rides first. A 2016 pooled analysis of five studies found that people who stopped driving had nearly twice the odds of depressive symptoms, though the studies couldn't show that stopping caused the depression.
- **Memory.** Frame it as a checkup: "Let's see if the doctor can help us figure out what's going on," and offer to go along. The Alzheimer's Association's free 24/7 Helpline is 800-272-3900.
- **Money.** Ask about safeguards, not amounts. Chapter 3's trusted-contact idea is a good first ask.
- **Home safety.** Tie every suggestion to staying home.

7. Talk with doctors without speaking over your parent. Plan together beforehand. Decide what issues to raise, who will raise them, and whether either of you wants time alone with the provider. Write the doctor a note in advance, let your parent and the doctor lead, and book a separate call or visit if you have more to say. Know that some patients won't speak honestly about sensitive subjects in front of a family member, and that some doctors will check partway through whether the patient is comfortable with you in the room. From Chapter 3, nothing stops you from telling a provider what you've noticed, even if they can't share anything back.

8. Choose language that preserves dignity.

Instead of	Try
"You need to..." / "You can't..."	"Would you be open to...?" / "I'm worried about..."
"You always..." / "You never..."	One specific, recent example
"I'm taking over."	"I'd like to help with X. You stay in charge of Y."
"Remember when I told you?"	Just say it again, calmly. Don't try to jog the memory.
"That's not true!" / "Don't be silly."	"That sounds upsetting. Let's look together."
"Good job, sweetie!"	Adult tone. Talking down can feel patronizing and infantilizing.

With accusations, ask yourself whether correcting is worth it. If your parent keeps accusing someone of taking a wallet, keep it in an easy-to-remember spot so you can show it's safe.

> **One family's story**
>
> At Thanksgiving, in front of everyone, Marcus tells Ruth she shouldn't be driving anymore. She leaves the table and doesn't speak to him for two days. In January, he tries again, privately and on the phone. He asks permission to bring it up, mentions the new dent in her car, and suggests a driving evaluation so she can prove him wrong. She agrees, because the decision is still hers.

4. Exactly what to say

Use this skeleton for any conversation:

"I [care about / want] ___. I've noticed ___. Would you be open to ___? Or would you rather ___?"

Worked example (money): "I want you to keep control of your money. I noticed a few bills went unopened. Would you be open to adding me as a trusted contact at the bank, so they call me if something looks like a scam? I wouldn't see your accounts. Or would you rather pick someone else?"

Worked example (driving): "I want you to keep your independence. I got nervous on Tuesday at the turn by the church. Would you be open to a driving evaluation, or would you rather start with Dr. Lee's opinion?"

The full toolkit is next.

5. Tool: Twenty-Five Difficult-Conversation Scripts

Opening

1. First conversation: "Mom, I'm not here to take anything over. I've noticed a few things, and I'd rather ask you than guess. Is now a good time?"

2. Bad timing: "It sounds like this isn't a good moment. Can we try Sunday after lunch?"

3. After a blowup: "I handled that badly. I care about you and I got pushy. Can we start over?"

When they refuse

4. "I'm fine." "I believe you want to be fine. I'd just feel better if we checked. What would be the easiest first step?"

5. "It's none of your business." "You're right that it's your life. I love you, so I worry. Can you tell me what part feels intrusive?"

6. "I don't need help." "I'm not saying you do. Could I help with one small thing, just so I stop worrying?"

7. "Stop treating me like a child." "You're right, and I'm sorry. How would you want me to bring things up?"

Driving

8. "Driving isn't what it used to be. Do you ever feel it's harder than it was?"

9. "Would you mind if I rode along Thursday? I'd like to see what you're seeing."

10. "I'd feel better with a professional evaluation. Would you do it for me? Or should we start with the doctor?"

Memory

11. "I've noticed we've both had some trouble remembering things lately. Let's see if the doctor can help us figure out what's going on."

12. "Would you let me come along? Two sets of ears are better than one."

13. If they deny it: "I hear you, and you may be right. A checkup will tell us either way."

Money

14. "I'm not asking how much you have. I'd just like to know where the papers are, in case something happens."

15. "There's a safeguard called a trusted contact. I'd have no access. Would you consider it?"

16. If a scam is suspected: "Before you send anything, would you call me first? It can be our rule for any request for money."

Home safety

17. "I want you to stay in this house. What would make it feel safer to you?"

18. "A therapist can check the home for free or at low cost. They look for trip hazards, not at you. Would you try it?"

Anger, accusation, repetition, suspicion

19. Anger: "I can see how upset you are. I'm going to step out for ten minutes and come back."

20. "You're stealing from me": "That sounds frightening. Let's look for it together." *(Don't argue.)*

21. Repetition: Answer the question again, in the same calm tone as the first time. Add a written note or calendar entry if it helps.

22. Suspicion of you or a sibling: "You have every right to ask questions. Shall we call the bank together and check?"

Doctors

23. Before the visit: "Is there anything you want me to bring up? Anything you'd rather say yourself?"

24. In the room: "Mom, you tell the doctor how you've been feeling, and then I'll add what I've noticed. Is that okay?"

25. Privately, afterward: "I want to share a concern without upsetting my mother. Could I send a note, or schedule a call?"

6. When professional help is appropriate

- **The doctor:** ask them to raise memory, driving, or medication concerns. Many parents will take it more seriously from a doctor than from a child.
- **Alzheimer's Association Helpline:** call the free 24/7 line, 800-272-3900, for scripts tailored to your situation.
- **Driving:** a doctor or occupational therapist can assess. If your parent refuses and is a danger, you can contact your local DMV, and some states accept anonymous reports while others don't.
- **Family conflict:** a geriatric care manager, social worker, or mediator can serve as the neutral voice.
- **Safety emergencies:** use Chapter 2. Suicidal talk means 988 or 911. Suspected abuse or exploitation means Adult Protective Services.
- **You:** if conversations leave you shaking or resentful, a counselor or caregiver group can help.

Pick one script and one topic this week. If it goes badly, write down what happened and try again.

Chapter 6: They Can Still Make Decisions—So When Should I Step In?

Days 22–25 of your 30-day plan

1. The frustration

"My parent is still independent in many ways, but some decisions are becoming risky."

Your mother still runs her book club and does her own taxes. She also has two bottles of the same pill on the counter, and last week she mentioned "a very nice man from the government" who needs her to buy gift cards. You don't have clear grounds to step in. Step in too early and you take her dignity. Wait too long and she may lose her savings, her health, or her license to someone else's mistake, or her own.

Nobody can tell you exactly where the line is. You can work it out one decision at a time, and the rest of this chapter shows how.

2. What may be happening

The starting point is respect. Capacity is decision-specific. Adults are presumed to have it, and an unwise choice is not proof it's missing. A patient with capacity can make decisions that lead to illness, discomfort, danger, or even death, and relatives often need education and support when that happens. So the question isn't "Is Mom competent?" It's "Can she understand, weigh, and communicate *this* decision?"

Money often shows trouble first. A 2020 study in JAMA Internal Medicine of about 81,000 Medicare beneficiaries 65 and older living alone found that people who later developed dementia were more likely to miss payments on credit accounts beginning about six years before diagnosis, and more likely to have subprime credit scores starting about two and a half years before. The differences were small, so one late bill proves nothing. Look for patterns.

Scam losses are growing. The FBI's 2025 figures show adults 60 and older reported $7.7 billion in losses, about a 60% increase from 2024. In August 2025 the FTC reported that, from 2020 to 2024, reports of losses of $10,000 or more by adults 60 and older to impersonators more than quadrupled. Payment method is a tell. The FTC has reported gift cards, bank transfers, and Bitcoin ATMs among the ways impersonation scammers get paid.

Suspicion isn't always dementia. Paranoia in older adults can come from medication side effects, hearing and vision problems, delirium, and dementia, and many causes are treatable. If it comes on suddenly, call for an urgent appointment that includes evaluation for a new illness or medication side effect.

Medication errors are easy to miss. Duplicates are easy to create when several doctors and pharmacies are involved, because the same drug can go by both a brand name and a generic name.

3. What to do now

Use the ladder, starting at the bottom. State laws generally allow guardianship only as a last resort and require consideration of less restrictive alternatives, and your approach should follow the same logic.

1. **Observe and log** (details below).
2. **Talk and offer.** Use Chapter 5's scripts.
3. **Add safeguards that leave decisions with your parent.** Examples are a bank trusted contact, one pharmacy, and a "call me first" rule.
4. **Make support formal while capacity remains.** Supported decision-making lets people keep making their own decisions with support from trusted individuals they choose. Powers of attorney (Chapter 3) work the same way.
5. **Get a professional assessment** for a specific decision.
6. **Go to court** only when everything else has failed, and only with an elder law attorney.

Topic playbooks

Concern	Signs	Gentle first step	Stronger step
Money and scams	Secret phone calls, urgent "government" demands, gift cards, wire or crypto requests, unopened bills, unusual purchases	"Call me before sending money to anyone who contacted you first." Add a trusted contact.	Bank fraud department, FTC report at ReportFraud.ftc.gov, or the National Elder Fraud Hotline (1-833-372-8311). The FTC says it will never demand money, make threats, or tell you to transfer money.

Concern	Signs	Gentle first step	Stronger step
Driving	Unexplained dents and scrapes, trouble following signals, needing help with familiar routes	Ride along, then use Chapter 5's scripts	A driver rehabilitation specialist can test driving on and off the road and help decide when to stop.
Medications	Duplicates, missed doses, confusion about pills	Brown-bag review (Chapter 3)	One pharmacy and synchronized refills; blister packaging
Suspicion	New accusations, fear of intruders	Rule out medical causes first. Don't argue.	Doctor visit within days, sooner if sudden

For any sudden, new suspicion, treat it as a Chapter 2 yellow. If there's violence, weapons, or a threat to anyone's safety, call 911.

Document without building a case. Your log (Chapter 2) exists to help professionals see patterns, not to prove your parent is incompetent.

- Record facts, dates, and what you saw, not labels like "paranoid."
- Include what's going *well*.
- Tell your parent: "I'm keeping a list of things to ask the doctor. Want to add anything?"
- Don't record conversations secretly. Federal law requires only one party's consent, but about 11 states generally require everyone's consent, so check your state's law. Secret recording also costs trust.
- Don't open mail, log into accounts, or move money without permission or legal authority.
- Share the log with doctors and your care team, not as ammunition in sibling disputes.

> **One family's story**
>
> Ruth wires $4,000 to a caller claiming to be her grandson in jail. She's embarrassed, and she insists she can still manage her own money. Marcus decides she's right: one bad decision doesn't mean she can't make decisions. Instead of taking over her accounts, he asks her to name him as a trusted contact at her bank and to set an alert for any withdrawal over $500. She keeps control, and when the next scam call comes, the bank catches it.

4. Exactly what to say

Money: "I'm not asking to see your accounts. I'd like us to have a rule: before you send money or gift cards to anyone who contacted you first, you call me. I won't say no. I just want a second set of ears."

Bills: "Would you let me be your backup? You'd keep paying them. The company would send me a copy, so nothing slips if you're ever sick."

Medications: "Could we ask the pharmacy to pack everything in a calendar card? Then you don't have to sort pills every week."

Suspicion about a missing item: "That sounds upsetting. Let's look together." *(Find it. Don't argue.)*

Suspicion about someone entering the house: "That sounds frightening. I want you to feel safe. Could Dr. Lee check whether a medicine or an infection could be making nights harder?"

When they say "I'm still independent": "You are, and I want to keep it that way. These two safeguards are so you can keep deciding."

To a sibling who sees your log as a case: "I'm keeping a log so Dr. Lee has facts. It isn't to prove anything about Mom."

5. Tool: The Independence and Risk Decision Matrix

Step 1: Rate the risk.

- **Low:** inconvenient and reversible (a messy kitchen, one late bill)
- **Moderate:** could cause harm over weeks or months but can be fixed (occasional missed pills, a near-miss while driving)
- **High:** serious or irreversible harm soon (payments to scammers, wrong or duplicated medicines, driving incidents)

Step 2: Note your parent's response. Do they accept help, feel ambivalent, or refuse (or lack insight)?

Step 3: Find the zone.

	Accepts	Ambivalent	Refuses / lacks insight
Low	🟢 *Respect.* Offer help.	🟢 *Respect.* Mention once, then log.	🟢 *Respect.* Log and revisit in 30 days.

	Accepts	Ambivalent	Refuses / lacks insight
Moderate	🟡 *Partner.* Add a safeguard this month.	🟡 *Partner.* Offer a choice of two safeguards. Involve the doctor.	🟡 *Watch and escalate.* Log, try LEAP, get the doctor involved.
High	🔴 *Act together this week.*	🔴 *Protect.* Use the least restrictive safeguard now and consult a professional.	🔴 *Protect now.* Doctor, professional assessment, and (as relevant) your bank, APS, the DMV, or an attorney.

Step 4: Complete the worksheet.

Concern	What I've seen (dates)	Risk	Parent's response	Zone	Lowest-rung step	Who / when	Review date

Step 5: Before I step in, ask:

- [ ] Have I ruled out a medical cause?
- [ ] Is this risky to them or someone else?
- [ ] Can it be undone?
- [ ] Have I tried the least restrictive step?
- [ ] Does my parent know what worries me?
- [ ] Is this a safety issue or only a difference in values?
- [ ] Has a doctor been involved?

6. When professional help is appropriate

- **The doctor:** start here to rule out medical causes. If cognitive impairment is suspected, Medicare covers a separate visit for a more detailed cognitive assessment and care plan. Medicare's description of that visit includes an evaluation of safety, including driving, and an assessment of the caregiver's needs and role.
- **A capacity evaluation:** a geriatrician, psychiatrist, or psychologist can assess capacity for a *specific* decision, such as managing money or signing a document.
- **Driving:** a Certified Driving Rehabilitation Specialist holds a credential from the Association for Driver Rehabilitation Specialists and is often an occupational therapist. The evaluation includes clinical and behind-the-wheel components, and may

recommend adaptive equipment, retraining, or stopping.
- **Finances:** the bank's fraud department; Adult Protective Services if you suspect exploitation; and an elder law attorney for powers of attorney or, as a last resort, guardianship.
- **The pharmacist:** for a medication review.
- **You:** if you're lying awake over this, a counselor or caregiver group can help.

Start with the log. Every other rung on the ladder builds on it.

Chapter 7: But Our Family Situation Is Different

Days 26–28 of your 30-day plan

1. The frustration

"Most advice assumes we live nearby, have money, agree with one another, and have a cooperative parent. We don't."

The advice reads fine until you try to use it. "Hold a family meeting," but your brother hasn't spoken to your sister in six years. "Visit weekly," but you live nine hours away. "Hire an aide," but a quick search shows the going rate, and you do the math. "Talk to your mom," but she hangs up on you.

Every article quietly begins with the same sentence: *If your parent is cooperative and you live close and the family gets along…* You're not that family. Almost nobody is. So the plan has to start from the family you actually have.

2. What may be happening

Generic advice fits an average family that doesn't exist. Your situation differs along five dials, and the combination changes everything: **distance, your parent's cooperation, how many hands you have, whether the issue is physical or cognitive, and money.**

Caregiving takes a toll. In AARP and the National Alliance for Caregiving's 2025 report, 63 million Americans provided care, nearly one in four of them 40 or more hours a week, and half of employed caregivers reported work disruptions. One in five caregivers reported fair or poor health. Your situation shapes how much of that falls on you. AARP's 2016 survey of caregiver costs found that long-distance caregivers had the highest out-of-pocket costs ($11,923 a year), and that caring for someone with dementia cost far more ($10,697 versus $5,758).

So aim for the **minimum workable level of support**, not the *ideal* plan: the smallest amount of help that keeps your parent safe and as independent as they want to be, and that your family can sustain for at least a year. A plan that burns you out in three months isn't a plan.

3. What to do now

Step 1: Locate yourself on the five dials. (The tool has a form.)

Step 2: Choose your path.

Dial	If this is you…	Do this	Skip this
Distance	**Local**	Set limits now. Many adult children slip into the primary-caregiver role by taking on one task at a time until they own everything.	Becoming the default for every task
Distance	**Long-distance** (an hour or more away)	Get your parent's written consent so providers can share information. Build a local network, consider hiring a local care manager, and join telehealth visits with permission. Use visits to give the local caregiver a break.	Trying to manage everything by phone
Parent	**Cooperative**	Make a shared plan and finish legal documents now (Chapter 3)	Waiting for a crisis
Parent	**Resistant**	Stay on the lower rungs of Chapter 6's ladder. Use Chapter 5's scripts. Prepare for a crisis in advance: Grab Sheet, doctor contact, backup people.	Pushing for a full plan at once

Dial	If this is you…	Do this	Skip this
Hands	**One caregiver**	Write the plan down even if you're alone. Ask anyone, even a distant cousin, for one specific task. Add paid help earlier than you think you need it.	Doing everything quietly until you break
Hands	**Several siblings**	The Family Caregiver Alliance says sibling discord most often comes from an unequal division of caregiving duties. Hold a meeting with an agenda and share feelings rather than accusations. If communication is contentious, add an outside facilitator such as a social worker, counselor, or religious leader. Divide tasks by skills, location, and schedules, not birth order.	Insisting on equal splits
Needs	**Mild physical limits**	Home safety changes (Chapter 3), an OT home evaluation, a personal emergency alert, rides, and meal help	Early institutional placement
Needs	**Cognitive concerns**	Act on legal documents and finances first (Chapters 3 and 6). Ask for the separate Medicare visit for detailed cognitive assessment and care planning. Plan for supervision, not just assistance.	Assuming it will plateau
Money	**Limited**	Start with free and subsidized help (below)	Giving away assets informally

Dial	If this is you…	Do this	Skip this
Money	**Flexible**	Still start small. Use a care manager to avoid overbuying.	Overbuilding before you know the need

For limited finances, work this list in order:

1. **Eldercare Locator (1-800-677-1116)** to reach your Area Agency on Aging. Through the National Family Caregiver Support Program, caregivers can get information, counseling, support groups, training, respite care, and limited supplemental services. What's offered, and who qualifies, varies by area.
2. **Medicaid.** In many states, Medicaid pays for non-medical home care and in-home supports for people who qualify. It has income and asset limits and may involve wait lists, so apply early.
3. **PACE**, a Medicare and Medicaid program for people 55 and older who need nursing-home-level care but can live safely in the community with its support. It's offered only in some states and areas.
4. **Veterans benefits**, if your parent served. The VA offers homemaker and home health aide care, and Aid & Attendance payments on top of a VA pension for qualified veterans.
5. **Be careful with assets.** In most states, Medicaid reviews transfers made in the five years before you apply and can impose a penalty period. Informal gifts and unwritten loans to family can cause trouble. See an elder law attorney first.

Step 3: Choose your level. These are national medians from CareScout's 2025 Cost of Care Survey. Local costs vary, and the weekly and yearly figures at Levels 2 through 4 are calculated from them.

Level	What it looks like	Rough cost
0	Calls, a neighbor who checks in	$0
1	Rides, delivered meals, pill organizer, emergency alert, mail check	Low

Level	What it looks like	Rough cost
2	Part-time paid help. The national median for a non-medical caregiver is $35 an hour, so 4 hours a week is about $7,280 a year and 10 hours about $18,200. Adult day health care is $95 a day, so two days a week is about $9,880.	$7,000 to $20,000+/yr
3	Daily help, about 44 hours a week	About $80,080 a year
4	Assisted living	$6,200 a month ($74,400 a year)
5	Nursing home or skilled care	$315 a day for a semi-private room ($114,975 a year); private duty nursing at home is $90 an hour

Pick the *lowest* level that passes three tests: **Is it safe? Can we sustain it for a year? Will my parent accept it?** Re-test every 90 days and after any red or yellow event. Adult day programs deserve a look because they remain one of the more affordable options while providing supervision, engagement, and structured daytime support.

Step 4: Build a local backup network, even from far away.

- **Eyes:** a neighbor, and Carrier Alert, a free service where letter carriers watch for mail piling up and alert a local agency, which calls your parent and then their emergency contact. It isn't offered everywhere, so ask the post office or the local letter carriers' branch.
- **Hands:** someone within an hour. Consider giving a trusted person a key or garage code, and keep a medical conditions and medication list on the fridge.
- **Rides and errands:** faith community, volunteers, Area Agency on Aging programs.
- **Professionals:** the doctor's office contact, the pharmacist, a care manager.
- **Emergency:** who has the Grab Sheet from Chapter 2.

Step 5: Know when free family care is no longer enough. Watch for these tripwires:

- **Safety:** your parent needs supervision at night, wanders, falls with no one around, or needs lifting or medical care you aren't trained for.
- **You:** you're providing 40 or more hours on top of a job, missing your own medical care, snapping at your parent, or dreading each day.
- **Money:** out-of-pocket costs are outrunning your budget.

- **Trend:** your parent keeps declining despite your effort, or has been hospitalized more than once.

Three or more means act this week. Start with respite. The ARCH National Respite Locator (archrespite.org) helps you find programs by location. Then add paid hours.

> **One family's story**
>
> Elena is worn out and resents Sofia, who sends money but rarely visits. Sofia feels shut out of decisions. During a tense video call, they stop arguing about what's fair and list what each of them is actually good at. Sofia takes over the insurance calls, the bills, and researching options from across the country. She also pays for a home aide two afternoons a week, so Elena gets regular time off.

4. Exactly what to say

Opening a sibling meeting: "I'm not asking for equal. I'm asking for specific. Here are six tasks. Which would you take?"

To a far-away sibling: "I know you can't be here. The most useful thing would be to own the insurance calls and bills, and to cover me one weekend a quarter."

To a sibling in denial: "I'm not trying to make you feel guilty. I'd like you to hear what the doctor said. Would you join the call Thursday?"

To your parent, about outside help: "This isn't replacing family. It's so I can keep being your daughter instead of your full-time aide. Can we try it for 30 days?"

To a neighbor: "Would you be one of three people I can call if something seems off? I'd only ask for a quick look."

To an agency: "Are your caregivers employees, and are they background-checked? What's the minimum shift? What are your rates for nights and weekends?"

To yourself: "Good enough and sustainable beats perfect and short-lived."

5. Tool: The Personalized Care Path Builder

A. My five dials (circle one in each)

Distance: Local / Long-distance | Parent: Cooperative / Resistant | Hands: One caregiver / Several | Needs: Physical / Cognitive | Money: Limited / Flexible

B. My three path adjustments (from the Step 2 table)

1. _____
2. _____
3. _____

C. My level

Level chosen: ___

- ☐ Safe?
- ☐ Sustainable for a year?
- ☐ Parent accepts?

Review date: ___ | Move *up* if: ___ | Move *down* if: ___

D. Backup Network Card

Eyes: ___ | Hands (within an hour): ___ | Rides: ___ | Pharmacist: ___ | Doctor's contact: ___ | Care manager: ___ | Holds Grab Sheet: ___

E. Money Map

Parent's income and assets: ___ | Siblings' agreed contributions: ___

Calls to make:

- ☐ Eldercare Locator
- ☐ State Health Insurance Assistance Program
- ☐ VA/Medicaid
- ☐ Elder law attorney

F. Free-Care Tripwires (check any that apply)

- ☐ Night supervision needed
- ☐ Wandering
- ☐ Lifting/medical tasks
- ☐ My health slipping
- ☐ 40+ hours a week
- ☐ Money strain
- ☐ Repeated hospitalizations

Three or more: call the Area Agency on Aging this week and ask about respite.

6. When professional help is appropriate

- **A geriatric care manager:** especially if you're far away. They're typically nurses or social workers who assess needs and coordinate local services, and they can be expensive but valuable.
- **An elder law attorney:** before gifts, transfers, or paying family members for care.
- **Your Area Agency on Aging:** for respite, programs, and benefits screening.
- **A mediator, social worker, or counselor:** when siblings can't talk without a fight.
- **A hospital discharge planner or social worker:** after any hospital stay.
- **Adult Protective Services:** if your parent is being neglected or exploited.
- **A counselor or support group for you.**

Choose your level, put the review date on the calendar, and call the Area Agency on Aging if you've checked three tripwires.

Chapter 8: Where Is the Insurance Card—and Who Paid That Bill?

Days 29–30 of your 30-day plan

1. The frustration

It's 6:40 p.m. and the ER nurse asks for the insurance card, the medication list, and the name of your mother's cardiologist. You're standing in her kitchen, opening drawers. The card isn't in her wallet. The medication list is a photo on your sister's phone. The cardiologist's name is on a business card that is "in the blue folder," which is not in the blue folder.

Neither of you did anything wrong. Information that worked fine while one person carried it in their head has to be findable by someone else, under pressure. About 63 million Americans, nearly one in four adults, provided care for a family member or friend in 2025, according to AARP and the National Alliance for Caregiving's *Caregiving in the US 2025*. Many learn this lesson mid-crisis. You're going to learn it now.

2. What may be happening

Three things are usually going on at once.

- **The records are scattered by design.** Insurers mail paper, doctors use portals, banks use apps, and each has its own login. Your parent may know exactly where everything is. That knowledge just isn't written down.

- **Nobody has been named.** Without authorization paperwork, offices can't talk to you. Without a central place, you can't hand the job to anyone else.
- **The money is mixed.** The first time you put a pharmacy bill on your own card "just this once," you start a record-keeping problem. It affects your parent's taxes, your own budget, and, later, your siblings' trust.

One caution before you gather everything in one place. The information that rescues you in an ER is also what a scammer wants. Medicare cards no longer carry a Social Security number; they carry a randomly generated Medicare Beneficiary Identifier. It still shouldn't be photographed and texted around.

3. What to do now

Day 29: build the core (about two hours).

1. **Print the binder** and start with page 2, the Emergency Quick Sheet. Print two copies: one for the fridge, one for your bag.
2. **Fill pages 3 through 5 from the real objects**, not from memory. Use the pill bottles for medications, the cards for insurance (copy front and back), and the business cards or portal pages for doctors.
3. **Use page 6 to record where the legal documents are.** Don't copy their contents. Keep copies in the binder and put the originals in a fireproof safe or with the attorney. A safe deposit box can be a poor home for a power of attorney. An agent may need the original to get into the box, and banks can restrict access after a death. Rules vary by state, so ask an elder law attorney.

Day 30: money, access, routines.

4. **Start the expense log (page 8) today.** The Consumer Financial Protection Bureau's guidance for agents under a power of attorney comes down to four duties: act only in your parent's interest, manage their money carefully, keep your money separate from theirs, and keep good records. Even without a power of attorney, the habit protects you. It also matters at tax time. If your parent is your dependent, you may be able to deduct, if you itemize, the part of the medical costs you paid that exceeds 7.5% of your adjusted gross income (IRS Publication 502).
5. **Fill the Accounts and Digital Access Index (page 10) as a map, not a vault.** Write where each login is kept and who can reach it, never the login itself. Put the actual passwords in a password manager with an emergency-access feature. Bitwarden (a paid Premium feature), LastPass, and Keeper offer a trusted contact who can request

access after a waiting period you set. As of this writing, 1Password's support pages describe a printed Emergency Kit instead of an emergency-access feature. These features change, so check each company's current plan details.

6. **Put a 10-minute monthly appointment on your calendar** for page 12. Part of that appointment is reading the Medicare Summary Notice. Original Medicare mails one every three months when it was billed, or monthly if your parent signs up for electronic notices. Look for any service your parent didn't receive. You can report suspicious billing to 1-800-MEDICARE (1-800-633-4227) or the Senior Medicare Patrol (1-877-808-2468).

What belongs where

Item	Paper (in the binder)	Secure digital storage	Neither
Quick Sheet, med list, contacts	✔	Photo of each page	
Insurance cards	Copy, front and back	Scan	
Legal documents	Copies and the location sheet	Scans	**Originals** go in a fireproof safe or with the attorney
Passwords, PINs		Password manager only	Never on paper in the binder, never by text
Receipts, statements	Current month	Scans, kept for taxes	

Sharing without exposing. Most people only need a slice of this information.

- Paramedics and the ER need page 2.
- A backup caregiver gets the whole binder.
- Everyone else gets last-4-digit references.
- Use official delegations instead of your parent's passwords: a signed HIPAA authorization at each doctor's office, patient-portal proxy access, and the bank's "trusted contact" or power-of-attorney form.
- Don't text photos of cards or email passwords.

> **One family's story**
>
> When Frank is taken to the ER after a dizzy spell, Dana stands at the registration desk unable to answer basic questions. She doesn't know his full medication list, his cardiologist's name, or where his Medicare card is. The next weekend she builds his emergency binder, and she keeps a one-page summary on his fridge and in her phone.

4. Exactly what to say

To your parent, introducing the binder: "Mom, I'm not trying to take anything over. I want one place with your doctors, your pills, and where the papers are, so that if something happens, nobody's guessing. You decide what goes in it and who gets to see it."

To your parent, about money: "When I pay something for you, I'll write it down and keep the receipt. We'll settle up on the first of the month so we never have to wonder."

To a doctor's office or bank: "I help my father with his care. What forms do you need on file so I can speak with you or see his statements? Can you add me as an authorized contact?"

To your siblings: "I've set up an emergency binder. Here's the one-page Quick Sheet for everyone. [Name] will hold a full backup copy. On the first of each month I'll send a short update from the log."

To a caller asking for a Medicare number: "Medicare doesn't call to ask for that. I'm hanging up and calling 1-800-MEDICARE."

5. Tool: Caregiver Emergency Binder

The companion PDF (see the appendix) has 12 pages. You can type into the fields on screen or print it and write by hand.

1. **Start Here:** how to use it, the privacy rule, and paper versus digital
2. **Emergency Quick Sheet:** the fridge copy
3. **Medications:** prescriptions, over-the-counter products, and supplements
4. **Doctors, Providers, and Pharmacy**
5. **Insurance and Benefits:** a Medicare Summary Notice review checklist and a claims tracker
6. **Legal Documents Location Sheet**
7. **Bills and Income**

8. **Expenses and Reimbursements Log**
9. **Backup Network, Home, and Service Providers**
10. **Accounts and Digital Access Index:** a map, not a vault
11. **Incident and Symptom Log:** the Chapter 2 triage log
12. **Monthly, Quarterly, and Yearly Maintenance Checklist**

6. When professional help is appropriate

- **An elder law attorney** when no power of attorney or health care directive exists, when your parent's capacity is changing (see Chapter 6), or for questions about trusts, property, or Medicaid planning.
- **A tax preparer** when you're paying a parent's medical costs and want to claim them correctly.
- **Your State Health Insurance Assistance Program (SHIP)** for free, unbiased Medicare counseling, especially during fall enrollment.
- **Adult Protective Services, and the police if money is missing,** if you suspect someone is exploiting your parent financially. Use the Senior Medicare Patrol for suspected Medicare fraud.

Chapter 9: I Don't Have Time to Read a Textbook

Day 30, and every week after

The two-minute version. If you read nothing else:

1. Fill in **one dashboard a week**. It takes ten minutes.
2. Give every task **one owner**, with a name and a date.
3. Fill in the **prep sheet** before every appointment.
4. When something is off, use the **decision trees**. Red means call 911, yellow means call the doctor today, and green means write it down.
5. Write the **handoff sheet** while things are calm, not on the worst day.

1. The frustration

You're in your car at lunch. The pharmacy has called, your sister has texted, the school has emailed, and you're scanning the index of a 300-page caregiving book for "what to do when Dad stops eating." You don't have time for a chapter. You need a page.

The load is real. In *Caregiving in the US 2025*, AARP and the National Alliance for Caregiving found that people caring for an adult 50 or older spend an average of 26 hours a week on it, up from 22 in 2020. Six in 10 family caregivers are employed, and half of those report work disruptions. Just over 20% have had any formal training on medical or nursing tasks, though over half manage them. You don't need more information. You need less of it, in the right place.

2. What may be happening

- **Books are organized by topic. Your problems arrive by urgency.** The answer exists, but you can't find it fast enough to use it.
- **The system is you.** Appointments, refill dates, and who promised what live in one head. That works until it doesn't.
- **You remake the same decisions every time.** Without rules, "Should I call the doctor?" gets decided from scratch at 9 p.m. each time.

What helps is a few pages that hold the information for you.

3. What to do now

The ten-minute weekly review (Dashboard, page 1). Pick a fixed time, like Sunday evening. Spend two minutes looking back (fill in *What changed?*), three looking ahead (appointments, refills), three assigning (every open task gets one owner), and two sharing (send a photo of the page to your backup and siblings).

Appointment prep on one sheet (page 2). The National Institute on Aging suggests ranking your concerns and planning on three or four, because that's what a visit can realistically cover. Bring the medicines (or the bottles), insurance cards, and other doctors' names. Tell anyone going with you what you want from the visit, and ask your parent whether they'd like time alone with the doctor. Before you leave, repeat the plan back in your own words.

Shared calendar and task ownership (page 3). Create one calendar named for your parent, and invite siblings and your backup person. Put every appointment on it with who is driving. Pick one place for messages and one for documents. Free tools can help, such as the care calendar at Lotsa Helping Hands. The real rule is on the page: whoever claims a task owns it until they hand it off out loud.

Tracking changes without writing a diary (Dashboard, *What changed?*). Check Better, Same, or Worse for each area, and add one line only when something is worse. Save full entries for yellow and red events (incident log, binder page 11). Log the change, not the

day.

Simple decision trees (pages 4 and 5). There are four:

- Something is different today.
- I can't reach my parent.
- A medicine problem.
- "I don't need help."

Stroke signs (BE-FAST: sudden trouble with Balance, Eyes, Face, Arms, or Speech) are a 911 call, even if the symptoms pass, and you should note when they started (American Stroke Association). When you're unsure, go one color up.

An emergency handoff another person can understand (page 6). Hospitals use structured handoffs for the same reason. In a 2014 *New England Journal of Medicine* study of structured handoffs (called I-PASS) across nine pediatric hospital residency programs, medical errors fell 23% and preventable adverse events fell 30%. Your parent's care isn't a hospital ward, but the same idea applies: use the same headings every time, and have the receiver say it back. The handoff sheet adapts the idea to five parts:

1. How stable things are
2. A short summary
3. A to-do list
4. If this happens, do this
5. A read-back

> **One family's story**
>
> Marcus and his sister used to talk about their mother in long, anxious phone calls that ended without a plan. Now they hold a 20-minute call every Sunday evening with a one-page dashboard: appointments, medication changes, new concerns, and who's doing what. Most calls end with three decisions and one owner for each.

4. Exactly what to say

To your siblings, setting up the weekly update: "Every Sunday I'll send a photo of a one-page dashboard. Read it, ask questions, and claim a task if you want one. If it's urgent, I'll call."

To a sibling, about ownership: "Which of these do you want to own? I'd rather you own one thing completely than help a little with everything."

To the doctor, at the start of the visit: "We have three things for today. The most important is that she's been dizzy when she stands up."

To your parent, about the handoff: "I'm writing down what a good day looks like for you, so if I'm ever away, whoever helps can keep things normal. What matters most about your mornings?"

To the person covering for you: "Here's the sheet. Read it back to me in your own words, and tell me what's unclear."

5. Tool: Weekly Care Dashboard and Emergency Handoff Sheet

The companion PDF (see the appendix) has six pages. You can type into the fields or print it and write by hand.

Page	Use it when	Time
1. Weekly Care Dashboard	Once a week	10 min
2. Appointment Prep and Visit Notes	Before and after each visit	5 min + 3 min
3. Who Owns What: Tasks and Shared Calendar	Once, then at monthly check-ins	20 min
4–5. Decision Trees	Something is off	2 min
6. Emergency Handoff Sheet	Before any absence, and every few months	30 min

6. When professional help is appropriate

- **Your Area Agency on Aging.** Call the Eldercare Locator (1-800-677-1116, call or text). Local agencies can arrange check-ins, meals, transportation, and help with medicines.
- **A geriatric care manager.** Hire one when the coordination outgrows the family, or when you live far away. They can also keep the plan current.
- **Your own doctor or a counselor.** Emotional stress among caregivers has risen since 2020, according to the AARP/NAC release. If you're skipping your own appointments, that's the signal.
- **Your parent's doctor,** to review the decision trees and add specifics for their conditions.

Chapter 10: What Happens When This Gets Worse?

Days 31–90, and beyond

The two-minute version.

1. **Stable** means the system works without you holding it together. Score it once a month.
2. Know your **triggers**. One trigger: tell your backup. Two in a month: reassess.
3. Add help **one step at a time**, not all at once.
4. In a hospital, ask on day one: **"Inpatient or observation?"**
5. Agree now on **the signs**. Decide the move later.
6. Put **your own** health, job, and limits on paper.

1. The frustration

It's Sunday night, and for the first time in a month nothing is on fire. You can't relax. Every phone call feels like the other shoe about to drop.

There's a reason for that. About one in four adults 65 and older reports a fall each year, according to the CDC, and the age-adjusted fall death rate for that group rose 21% from 2018 to 2024. This chapter starts where the 30-day plan ends. You can't prevent the next crisis, but you can make it smaller and less frightening.

2. What may be happening

- **The first 30 days fixed the emergency, not the direction.** Stability now means the plan is holding, not that things have stopped changing.
- **Change usually arrives as a step, not a slope.** A fall, a hospital stay, or a spouse's death can change everything in a day.
- **Families decide in the crisis.** Deciding ahead can feel like betraying a parent. So the first conversation happens in a hospital hallway.

The fix is to decide the signs now and the move later.

3. What to do now

What stability looks like (Day 30). Run the eight-point Stability Check (page 1). Examples: medicines taken as planned, no falls or ER visits this month, every ride has an owner, a backup could run a day from the handoff sheet, and you're keeping your own appointments. A score of 7 to 8 is stable. A score of 4 or lower means re-running the Chapter 3 assessment this week.

Changes that trigger a new assessment. The page lists ten, including a hospital stay or fall, new confusion, a bill or scam problem, a helper who quits, and you being exhausted most days. One trigger: update the dashboard and tell your backup. Two in a month: re-run Chapter 3, then the Chapter 7 Care Path Builder.

Increasing help at home. Climb a step only when a trigger says so: tools, then family on a schedule, then a few paid hours, daily help, around-the-clock care, and finally a move. CareScout's 2025 median is $35 an hour. Ten hours a week is about $18,200 a year. Medicare covers part-time skilled home health if your parent is homebound and a doctor orders it. It doesn't cover around-the-clock care, home-delivered meals, or personal care as the only need. If your parent has dementia and Original Medicare, ask about Medicare's GUIDE program, which includes a care navigator, a 24/7 helpline, and up to $2,500 a year in respite. Not every area has a participating provider.

Hospital and rehab transitions. Ask on day one whether your parent is an inpatient or in observation. Observation days don't count toward the three inpatient days Medicare requires for skilled nursing facility coverage. Hospitals should give an "Important Message from Medicare" within two days of admission. If you believe a discharge is too soon, you can request a fast appeal, no later than the planned discharge day. In 2026, Medicare's skilled nursing facility coverage costs you $0 for days 1–20 (after the $1,736 Part A deductible, if you haven't already paid it for that stay), then $217 a day for days 21–100. In older federal data from 2010, nearly one in five Medicare patients discharged from a hospital were readmitted within 30 days, and rates vary by hospital, so use the roadmap's first-72-hours list.

When living alone may no longer be safe. Page 6 has a ten-item check. Two or three items: step up help and ask the doctor for an assessment. Four or more, or any one of these, meet this week: can't get up after a fall, wandering, or a fire risk.

Comparing your options.

Option	Typical cost (national medians; CareScout 2025, A Place for Mom 2026)	Who may pay
Home care	$34–$35 an hour	Private pay, insurance; Medicare for part-time skilled care only
Assisted living	About $5,400–$6,200 a month	Private pay, insurance; Medicaid varies by state

Option	Typical cost (national medians; CareScout 2025, A Place for Mom 2026)	Who may pay
Memory care	About $6,690 a month	Same; Medicare doesn't cover custodial care
Nursing home	$315 a day semi-private ($114,975 a year)	Medicare for short rehab; Medicaid if eligible
Family care	About 26 hours a week; about $7,200 a year out of pocket	You

Preparing without deciding too soon. Tour two places and collect fee schedules and contracts, but pay no deposits. Don't sell the house or retitle assets without an elder law attorney. Don't promise "never a facility" or "always home."

Protecting your health, work, and family. The Alzheimer's Association reported in November 2025 that 59% of dementia family caregivers rate their emotional stress as high or very high. The federal Family and Medical Leave Act (FMLA) allows eligible employees up to 12 workweeks of unpaid, job-protected leave in a 12-month period to care for a parent with a serious health condition, in blocks or on a reduced schedule. To qualify, you generally need 12 months and 1,250 hours with an employer that has 50 or more employees within 75 miles of your worksite. Schedule respite, and put your limits in writing on page 7.

Building the 30-, 60-, and 90-day plan. Days 1–30: stabilize and test. Days 31–60: strengthen (trial help, tours, attorney, triggers). Days 61–90: reassess and decide at a second family meeting.

> **One family's story**
>
> Carmen falls and breaks her hip. In the hospital, the sisters are told she'll go to rehab, but nobody says what happens after that. They ask to meet the discharge planner early, tour two rehab options, and use the 90-day plan to decide, together with Carmen, how much help she'll need when she comes home.

4. Exactly what to say

To the hospital: "Is my mother admitted as an inpatient or in observation? When was that decided? I'd like to speak with the case manager today about discharge."

If discharge feels too soon: "I don't think she's safe to go home yet, because she can't get to the bathroom alone. I'd like her Important Message from Medicare and instructions for a fast appeal."

To your parent, about more help: "I'm not trying to take over. I'm trying to keep you home. Let's try a few hours a week for a month and see how it goes."

To your siblings: "Let's agree now on what would make us change the plan, so we're not arguing in a hospital hallway."

To your employer: "I'm caring for a parent with a serious health condition. Can we talk about FMLA or a flexible schedule?"

To your family, about respite: "I need one weekend off every quarter. Who can cover?"

5. Tool: Care Transition Roadmap and 90-Day Family Action Plan

The companion PDF (see the appendix) has nine pages. You can type into the fields or print it and write by hand.

1. **Stability Check and Re-Assessment Triggers**
2. **Increasing Help at Home**
3. **Roadmap 1:** During the Hospital Stay
4. **Roadmap 2:** Rehab and the First 30 Days Home
5. **Care Options Compared** (landscape)
6. **Prepare Without Deciding Too Soon:** living-alone safety check and decision triggers
7. **Protect the Caregiver**
8. **90-Day Family Action Plan**
9. **Family Meeting Agenda** (60 minutes)

6. When professional help is appropriate

- **Your parent's doctor and pharmacist:** a fall-risk review, a memory check, and a full medicine review.
- **The hospital case manager or social worker,** from day one. Use the quality reviewer (QIO) for a fast appeal.
- **An elder law attorney,** before moving assets, applying for Medicaid, or signing a facility contract.
- **A geriatric care manager or your Area Agency on Aging** (Eldercare Locator, 1-800-677-1116) when coordination outgrows the family.

- **Your own doctor or a counselor,** and in the US, 988 at any hour if caregiving ever feels hopeless.

You won't prevent every crisis. Keep the binder current, keep the roles clear, and keep your own health on the list, so the next one finds you better prepared.

Appendix: Your tools at a glance

Every chapter ends with one working tool. The tools for Chapters 1–7 are printed inside the chapters, so you can copy them onto paper or into your notes app. The tools for Chapters 8–10 are longer, so they come as fillable PDFs that go with this book.

Chapter	Tool	Where to find it
1	My Role and Responsibilities Worksheet	In the chapter
2	72-Hour Family Triage Checklist and Incident and Symptom Log	In the chapter
3	Seven-Area Aging Parent Assessment	In the chapter
4	Concern-to-Action Planner	In the chapter
5	Twenty-Five Difficult-Conversation Scripts	In the chapter
6	Independence and Risk Decision Matrix	In the chapter
7	Personalized Care Path Builder	In the chapter
8	Caregiver Emergency Binder	PDF, 12 pages
9	Weekly Care Dashboard and Emergency Handoff Sheet	PDF, 6 pages
10	Care Transition Roadmap and 90-Day Family Action Plan	PDF, 9 pages (page 5 is landscape)

The three companion PDFs

Caregiver_Emergency_Binder.pdf (Chapter 8, Days 29–30). Twelve pages: Start Here, Emergency Quick Sheet, Medications, Doctors and Pharmacy, Insurance and Benefits, Legal Documents Location Sheet, Bills and Income, Expenses and Reimbursements Log,

Backup Network, Accounts and Digital Access Index, Incident and Symptom Log, and the Monthly, Quarterly, and Yearly Maintenance Checklist.

Weekly_Care_Dashboard_and_Emergency_Handoff.pdf (Chapter 9, Day 30 and every week after). Six pages: Weekly Care Dashboard, Appointment Prep and Visit Notes, Who Owns What, two pages of Decision Trees, and the Emergency Handoff Sheet.

Care_Transition_Roadmap_and_90_Day_Plan.pdf (Chapter 10, Days 31–90 and beyond). Nine pages: Stability Check and Re-Assessment Triggers, Increasing Help at Home, two hospital and rehab roadmaps, Care Options Compared, Prepare Without Deciding Too Soon, Protect the Caregiver, the 90-Day Family Action Plan, and a 60-minute Family Meeting Agenda.

How to use them. Type into the fields on screen, or print the pages and write by hand. Some web browsers display a PDF but won't save what you type, so if your entries disappear when you close the file, print it or open it in a desktop PDF reader. Keep a printed copy of the Emergency Quick Sheet on the fridge and another in your bag.

Sources and As-Of Dates

Every figure in this book that came from a website, report, or study is listed here by chapter. Each entry gives the publisher, the date of the source or the period its data covers, and a web address where one is available. All web sources were checked on November 1, 2026. Figures, program rules, and web addresses change, so check the current version before you rely on a number. Costs and programs also vary by state and by community.

Chapter 1

- **Caregiver numbers and strain** (63 million caregivers, nearly one in four adults; 59 million caring for an adult; up about 20 million since 2015; nearly one in four giving 40 or more hours a week; one in five in fair or poor health; nearly half with a major financial impact). AARP and National Alliance for Caregiving, *Caregiving in the US 2025*. AARP news release, July 24, 2025 and AARP report page.
- **Falls** (one in four adults 65 and older fall each year; leading cause of injury and injury death; age-adjusted fall death rate up 21% from 2018 to 2024). CDC, Older Adult Falls Data and Research, page reviewed September 4, 2026, data through 2024.

- **Fall injuries and telling a provider** (about 37% injured; about half did not discuss the fall with a provider). CDC, MMWR, September 23, 2016, "Falls and Fall Injuries Among Adults Aged ≥65 Years — United States, 2014". Data year 2014.
- **Delirium** (comes on over days; dementia over months to years; infections and medications are common causes). VA Geriatrics and Extended Care, What Is Delirium? and What Causes Delirium?, both updated July 29, 2022.
- **Care costs** ($35 an hour; assisted living $74,400 a year; semi-private nursing home room $114,975 a year). CareScout, 2025 Cost of Care Survey results, released March 2, 2026. Data collected July to November 2025. National medians.
- **Medicare and long-term care.** Medicare.gov, Long-term care.
- **Eldercare Locator, 1-800-677-1116.** Eldercare Locator, Administration for Community Living, page modified March 2, 2026.

Chapter 2

- **Heart attacks without chest pain after age 75.** American Heart Association scientific statement, *Circulation*, December 12, 2022. AHA statement highlights.
- **Urinary tract infections and sudden confusion.** Cedars-Sinai, How UTIs Can Cause Delirium and Worsen Dementia.
- **Sepsis in adults 65 and older** (13 times more likely to be hospitalized; 2008 data). Sepsis Alliance, Sepsis and Aging fact sheet, revised February 2024, citing CDC National Center for Health Statistics, Data Brief 62, 2011.
- **Medication-related emergency hospitalizations** (about 99,600 a year; four drug types account for 67%; about 66% unintentional overdoses). Budnitz et al., *New England Journal of Medicine*, November 24, 2011, covering 2007–2009 data from 58 hospitals. Study summary, AHRQ PSNet and CDC news release, November 23, 2011.
- **Stroke warning signs (BE-FAST).** American Stroke Association, Stroke symptoms, and American Heart Association news release, May 4, 2026.
- **Poison Help, 1-800-222-1222.** HRSA, Poison Help, page modified January 8, 2025.
- **National Elder Fraud Hotline, 1-833-372-8311** (weekdays, 10 a.m. to 6 p.m. Eastern). U.S. Department of Justice, COPS Dispatch, September 2025 and Office for Victims of Crime.
- **988 Suicide and Crisis Lifeline** (24 hours a day; family and friends can call or text for guidance). 988 Lifeline.
- **Suicide rates** (men 75 and older, 40.1 per 100,000 in 2024). CDC National Center for Health Statistics, Data Brief 572, published September 23, 2026. Final 2024 data.

Chapter 3

- **Privacy rule and family.** U.S. Department of Health and Human Services, HIPAA: Family Members and Friends, page reviewed December 23, 2022 and modified March 16, 2026.
- **Medication counts and reviews.** University of Michigan National Poll on Healthy Aging, Older Adults' Experiences with Comprehensive Medication Reviews, October 2020 poll of 2,048 adults 50 to 80.
- **Home hazards.** CDC, Check for Safety: A Home Fall Prevention Checklist for Older Adults (archived copy). The archived page is undated.
- **Older drivers** (about 9,100 killed in 2022, 25 a day). CDC, Older Adult Drivers, page reviewed January 28, 2026. Insurance Institute for Highway Safety, Older drivers, updated July 2026.
- **Elder fraud losses** ($7.7 billion reported by people 60 and older in 2025, about 60% above 2024). FBI Internet Crime Complaint Center figures as reported in AARP, April 16, 2026. I could not open the FBI report itself, so this entry relies on the AARP article.
- **Trusted contacts.** FINRA, Why You Should Consider Adding a Trusted Contact to Your Account, July 15, 2025. FINRA's rule covers brokerage accounts; banks have their own practices.
- **Advance directives** (36.7% of U.S. adults; 45.6% of those 65 and older). Yadav et al., *Health Affairs*, July 2017, a review of 150 studies from 2011 to 2016. Study record.
- **Medicare annual wellness visit.** Medicare.gov, Yearly wellness visits. The page is undated.
- **Medicare cognitive assessment and care plan visit** (also Chapters 6 and 7). Summary of Medicare billing code 99483, document dated June 2024. Elements include a safety evaluation (including motor vehicle use), caregiver assessment, medication review, and advance care planning.

Chapter 4

- **Forgetting medical information** (40 to 80% forgotten almost immediately; almost half of what is remembered is incorrect). Kessels, "Patients' memory for medical information," *Journal of the Royal Society of Medicine*, May 2003. Full text.
- **Specific plans and follow-through** (94 independent tests). Gollwitzer and Sheeran, *Advances in Experimental Social Psychology*, volume 38, 2006, pages 69–119. Publication record.

- **Referrals that were not completed** (about 35% completed). "Closing the Referral Loop," *Journal of General Internal Medicine*, 2018. Data from July 2015 to June 2016 at one large health system. Study record.
- **Preparing for appointments.** National Institute on Aging, *Talking With Your Doctor* (2014 edition), reproduced by Family Caregiver Alliance.
- **Referrals in Original Medicare and Medicare Advantage.** Medicare.gov, Compare Original Medicare and Medicare Advantage. The page is undated.
- **UnitedHealthcare referral change.** UnitedHealthcare provider news, September 30, 2025. Effective January 1, 2026. This is one company's policy and may change.
- **State Health Insurance Assistance Program, 1-877-839-2675.** SHIP National Technical Assistance Center, page modified October 6, 2025.

Chapter 5

- **Preference to stay in the current home** (75% of adults 50 and older). AARP, 2024 Home and Community Preferences Survey, released December 10, 2024. Survey of 3,090 adults fielded June 17 to July 1, 2024.
- **Starting the conversation early.** Alzheimer's Association, 10 Steps to Approach Memory Concerns in Others.
- **Talking to a loved one about seeing a doctor** (44%). Alzheimer's Association and Ad Council survey, released November 1, 2021. 2,024 U.S. adults surveyed October 13 to 16, 2021.
- **Elderspeak and rejection of care.** "Rejection of Care in Hospitalized Persons Living With Dementia: The Impact of Nurse Communication," *Innovation in Aging*, 2021. 16 patients, 53 staff members, one hospital. Study record.
- **Driving cessation and depressive symptoms.** Chihuri et al., "Driving Cessation and Health Outcomes in Older Adults," *Journal of the American Geriatrics Society*, 2016. A pooled analysis of five studies. Study record.
- **Alzheimer's Association Helpline, 800-272-3900** (24 hours a day). Alzheimer's Association news release, November 3, 2025.
- **About the methods.** The descriptions of motivational interviewing and the LEAP method are summaries of how those methods work, not statistics.

Chapter 6

- **Financial warning signs before a dementia diagnosis.** Nicholas et al., *JAMA Internal Medicine*, published online November 30, 2020. Medicare beneficiaries 65 and older living alone. University of Michigan summary.
- **Impersonation scams** (more than fourfold increase from 2020 to 2024 in reports of losses of $10,000 or more by adults 60 and older). Federal Trade Commission, news release, August 7, 2025.
- **Gift cards as a payment method used by scammers.** Federal Trade Commission, news release, December 21, 2020.
- **What the FTC will never do** (demand money, make threats, tell you to transfer money). Federal Trade Commission, July 2023.
- **Recording conversations.** Reporters Committee for Freedom of the Press, Reporter's Recording Guide. Federal law requires one party's consent. About 11 states generally require all parties' consent. State laws change.
- **Certified Driving Rehabilitation Specialists.** Association for Driver Rehabilitation Specialists, What is a CDRS? and Who Provides Driver Rehabilitation Services.
- **Capacity and guardianship.** These sections describe general legal concepts, not statistics. State laws differ, so ask an elder law attorney.

Chapter 7

- **Caregiver strain.** AARP and National Alliance for Caregiving, *Caregiving in the US 2025* (see Chapter 1).
- **Caregiver out-of-pocket costs** ($11,923 a year for long-distance caregivers; $10,697 for dementia care versus $5,758). AARP, *Family Caregiving and Out-of-Pocket Costs: 2016 Report*. AARP infographic. 2016 data.
- **Sibling conflict.** Family Caregiver Alliance, Caregiving and Sibling Relationships, page modified October 2, 2026.
- **National Family Caregiver Support Program.** Administration for Community Living, page modified March 18, 2026.
- **PACE.** Medicare.gov, PACE. The page is undated. PACE is offered only in some states.
- **Medicaid look-back period** (60 months in most states). Medicaid Long-Term Care, Look-Back Period, updated December 23, 2025. California and some other states differ.
- **VA benefits.** VA, Aid and Attendance benefits and Housebound allowance, updated May 8, 2026. VA, Homemaker and Home Health Aide Care, updated March 6, 2026.

- **Care costs by level.** CareScout, 2025 Cost of Care Survey (see Chapter 1). The weekly and yearly figures at Levels 2 through 4 are calculated from the national medians (the same survey reports adult day health care at $95 a day, a semi-private nursing home room at $315 a day, and private duty nursing at home at $90 an hour): $35 × 4 hours × 52 weeks is $7,280; $35 × 10 × 52 is $18,200; $95 × 2 days × 52 is $9,880; $35 × 44 × 52 is $80,080.
- **Carrier Alert.** National Association of Letter Carriers, Carrier Alert. Not offered everywhere.
- **Respite locator.** ARCH National Respite Network, page modified October 20, 2023.

Chapter 8

- **Number of caregivers.** AARP and National Alliance for Caregiving, *Caregiving in the US 2025* (see Chapter 1).
- **Medicare card number.** Commonwealth of Massachusetts, New Medicare Cards, October 27, 2017. The card carries a randomly assigned Medicare Beneficiary Identifier instead of a Social Security number.
- **Duties of an agent under a power of attorney.** Consumer Financial Protection Bureau, Managing Someone Else's Money, March 2020.
- **Medical expense deduction** (7.5% of adjusted gross income; dependent rules). IRS Publication 502, page reviewed October 16, 2026. Tax rules change each year, so ask a tax preparer.
- **Password manager emergency access.** Company support pages for Bitwarden, Keeper, LastPass, and 1Password, accessed November 1, 2026. These features and plans change often.
- **Medicare Summary Notices and reporting fraud** (1-800-633-4227; Senior Medicare Patrol, 1-877-808-2468). AARP, Reporting Medicare Fraud, May 20, 2023.

Chapter 9

- **Weekly hours, work, and training** (26 hours a week for adults 50 and older, up from 22 in 2020; six in 10 employed, half with work disruptions; just over 20% with formal training; stress up since 2020). AARP and National Alliance for Caregiving, *Caregiving in the US 2025*. AARP news release, July 24, 2025 and AARP report page.
- **Appointment planning.** National Institute on Aging, *Talking With Your Doctor* (see Chapter 4).

- **Structured handoffs** (medical errors down 23%; preventable adverse events down 30%). Starmer et al., *New England Journal of Medicine*, November 2014, nine pediatric residency programs. Study summary, AHRQ PSNet and Intermountain Health.
- **Backup planning.** Family Caregiver Alliance, Smart Caregiving: Backup Planning and What If Something Happens to Me?.
- **Shared care calendar.** Lotsa Helping Hands.

Chapter 10

- **Falls.** CDC, Older Adult Falls Data and Research (see Chapter 1).
- **Home care, assisted living, and nursing home costs.** CareScout, 2025 Cost of Care Survey (see Chapter 1).
- **Assisted living, memory care, and in-home care costs** (median $5,419 a month for assisted living; $6,690 a month for memory care; $34 an hour for in-home care). A Place for Mom, senior care costs, news release, February 19, 2026. These reflect what families actually paid, which is why they differ from CareScout's survey prices.
- **Medicare 2026 amounts** (Part A deductible $1,736 per benefit period; skilled nursing facility coinsurance $217 a day for days 21 to 100). CMS, 2026 Medicare Parts A and B Premiums and Deductibles, November 14, 2025. These change every January.
- **Skilled nursing facility coverage** (three-day inpatient stay; observation days do not count; $0 for days 1 to 20). Medicare.gov, Skilled nursing facility care.
- **Home health coverage.** Medicare.gov, Home health services.
- **Observation status.** Medicare Rights Center, observation status fact sheet, 2023.
- **Fast appeal of a hospital discharge.** Medicare.gov, Getting a fast appeal in a hospital.
- **GUIDE dementia care program.** CMS, GUIDE Model FAQs, page modified October 27, 2026 (care navigator, 24/7 support line, respite up to $2,500 per patient per year).
- **Hospital readmissions** (nearly one in five Medicare fee-for-service patients readmitted within 30 days). Agency for Healthcare Research and Quality, HCUP Statistical Brief #89, April 2010. This is older data, and current rates vary by hospital.
- **Family caregiver out-of-pocket spending** (about $7,200 a year). AARP news release, July 24, 2025, citing *Caregiving in the US 2025*.
- **Stress among dementia caregivers** (59% rate it high or very high). Alzheimer's Association news release, November 3, 2025.
- **Family and Medical Leave Act.** U.S. Department of Labor, Fact Sheet #28: The Family and Medical Leave Act, revised March 2025.

Figures to recheck each year

- Medicare deductibles and coinsurance, each January.
- CareScout and A Place for Mom cost surveys, each spring.
- FBI and FTC fraud totals, each spring and summer.
- Employer, state, and Medicaid rules, including FMLA eligibility, recording laws, and look-back periods.
- Insurer referral rules, password manager features, and phone numbers.